



Eye Floaters

The complete family guide

What they are • Why they move between eyes • When they are dangerous
Treatments • Vitamins & supplements (the honest evidence) • Everyday tips

START HERE

Read this first

If ANY of these happen, get seen the SAME DAY

- A **sudden shower** of new floaters (like pepper, soot or a swarm of dots)
- **Flashes of light** (like a camera flash or lightning streaks), especially at the edge of vision
- A **dark shadow or curtain** creeping in from the side, top or bottom
- **Sudden blurring or loss of vision** in either eye

Where to go: an urgent optometrist/optician appointment, an eye casualty department, or A&E / the emergency room. In the UK you can also call NHS 111. Do not wait to see if it settles: these can be signs of a retinal tear or detachment, which is very treatable when caught early and can threaten sight if left.

The short version, in plain English

- Floaters are tiny clumps inside the clear jelly (the **vitreous**) that fills the eye. You don't see the clump itself; you see its **shadow** on the back of the eye.
- They are **extremely common**, especially after about 50, and in short-sighted people. Most are harmless.
- The jelly in **both eyes ages at roughly the same rate**. So it is very normal for floaters to appear in one eye, then months later in the other. That fits your dad's story.
- Floaters usually become **much less noticeable over 3 to 12 months** because the brain learns to ignore them and they sink out of the line of sight. They rarely vanish completely, so they can seem to 'come back' when light, tiredness or attention change.
- The main risk is a **retinal tear** at the time floaters first appear. That is why **every new episode deserves a dilated eye check**, even if the last one was fine.
- Floaters that keep coming and going, especially with **red, painful or light-sensitive eyes or blurry vision**, can also be caused by **inflammation (uveitis)**, which needs treatment. Only an eye exam can tell the difference.
- No vitamin or eye drop has been **proven** to dissolve floaters. A couple of small studies are interesting (page 10), but none are strong. Laser and surgery exist for severe cases.

What we'd suggest for your dad right now

1. Book a **dilated eye examination** with an optometrist (optician) or ophthalmologist, and mention the pattern: right eye for ~9 months, then left for ~9 months, now back again.
2. Ask specifically: *"Is this a posterior vitreous detachment? Is there any tear, bleeding or inflammation?"* (more questions on page 13).
3. Learn the red-flag symptoms above, and go urgently if they appear.
4. Only start supplements after checking with his GP or pharmacist, especially if he takes blood thinners or heart medicines.

CHAPTER 1

What are floaters?

Floaters are small shapes that drift across your field of vision. They look like dots, threads, squiggles, cobwebs or rings. They are easiest to see when looking at something bright and plain, like a white wall, a computer screen or a blue sky. When you try to look straight at one, it darts away, because it is **inside** your eye and moves when your eye moves.

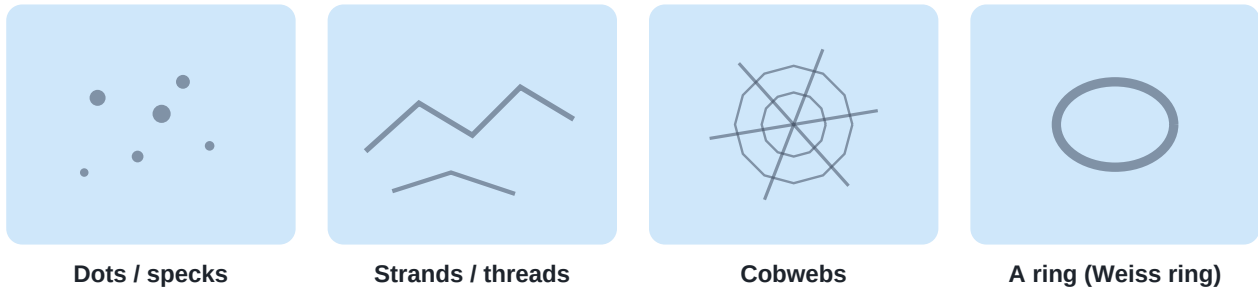


Figure 1. Common shapes people describe. A ring shape often appears after the gel separates from the back of the eye (see Chapter 3).

How the eye works (and where floaters live)

Think of the eye as a camera. Light passes through the **cornea** (the clear front window) and the **lens**, which focus it onto the **retina**, a thin layer of light-sensing cells lining the back of the eye. The retina sends the picture down the **optic nerve** to the brain.

Between the lens and the retina is a large space filled with a clear jelly called the **vitreous**. It is about 99% water, held in shape by a fine mesh of **collagen fibres** and a slippery molecule called **hyaluronic acid**. When fibres in that jelly clump together, they block a little of the light, and cast a **shadow on the retina**. That shadow is what you see as a floater.

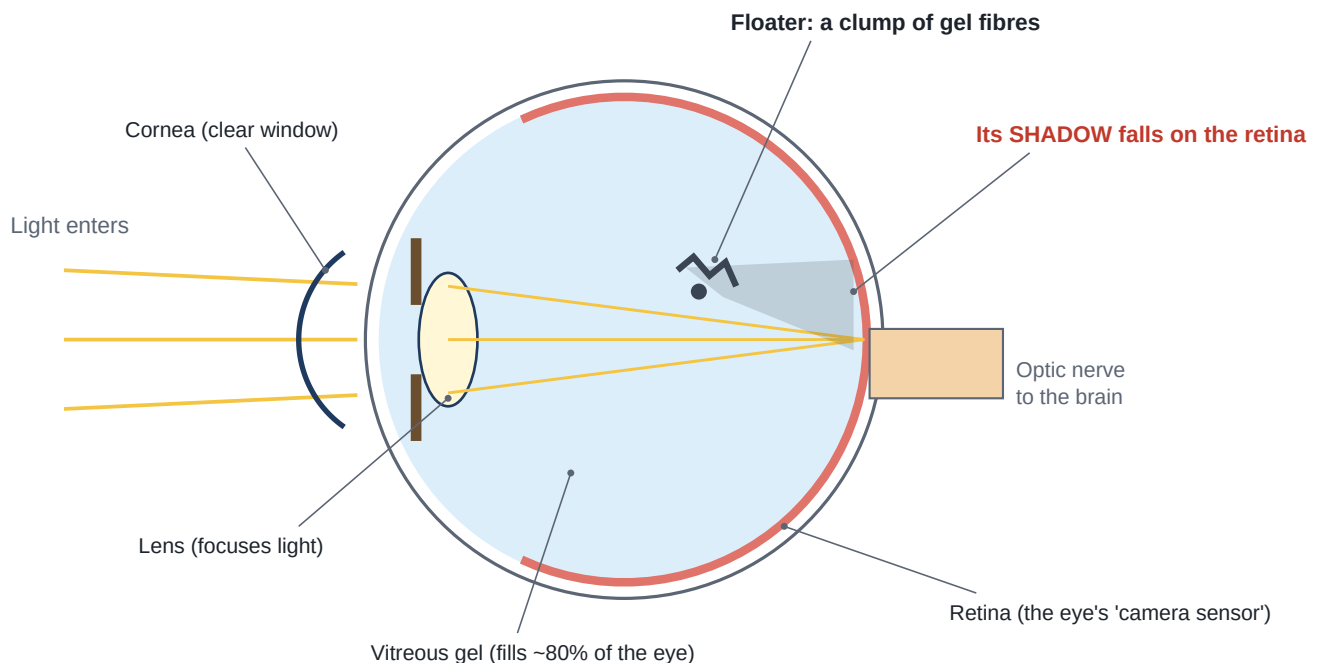


Figure 2. A cross-section of the eye. The floater sits in the gel; what you actually 'see' is the shadow it throws onto the retina.

CHAPTER 2

Why do floaters happen?

In most adults, floaters are a normal part of ageing. When we are young, the vitreous is a firm, clear gel. From our 40s onwards it slowly **liquefies** (doctors call this **syneresis**). Watery pockets form, and the collagen fibres that used to be evenly spread out collapse and bundle together into strands and clumps.

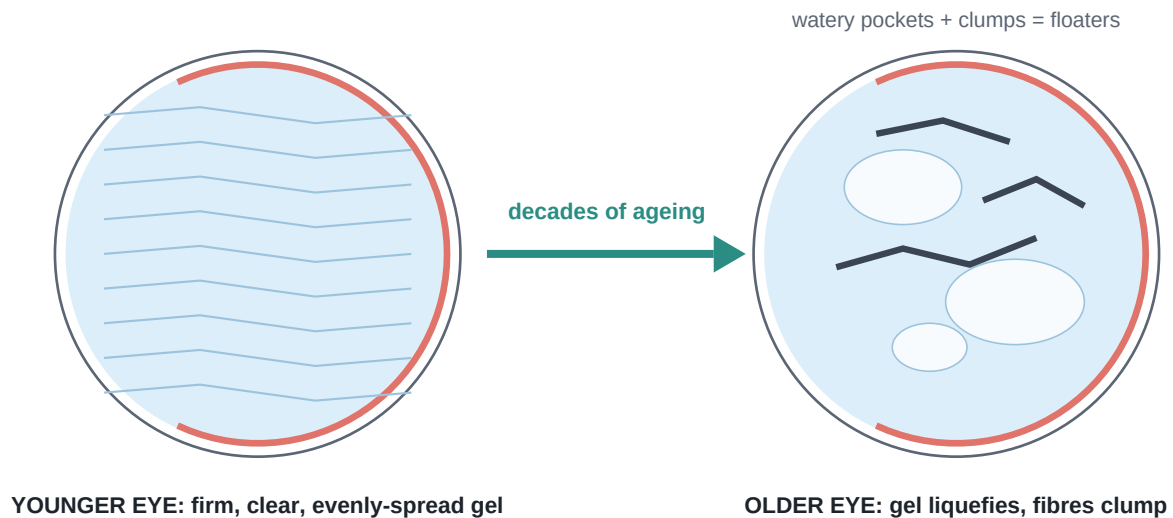


Figure 3. Over decades the gel turns partly to liquid, and loose fibres gather into visible clumps.

Things that make floaters more likely

Factor	Why it matters
Age (50+)	The gel naturally liquefies and shrinks with age. This is by far the most common cause.
Short-sightedness (myopia)	A longer eyeball stretches the gel, so changes start earlier, sometimes in the 30s.
Previous eye surgery	Cataract surgery and some laser treatments can speed up changes in the gel.
Eye injury	A knock to the eye or head can cause bleeding or gel separation.
Diabetes or high blood pressure	Fragile blood vessels in the retina can bleed into the gel, which looks like floaters or 'smoke'.
Inflammation (uveitis)	White blood cells and debris in the gel can look like floaters. Often linked to autoimmune conditions or infection.
Migraine-type visual symptoms	Zig-zag lights that last 20-30 minutes are usually from the brain, not the eye, but are worth mentioning to a doctor.

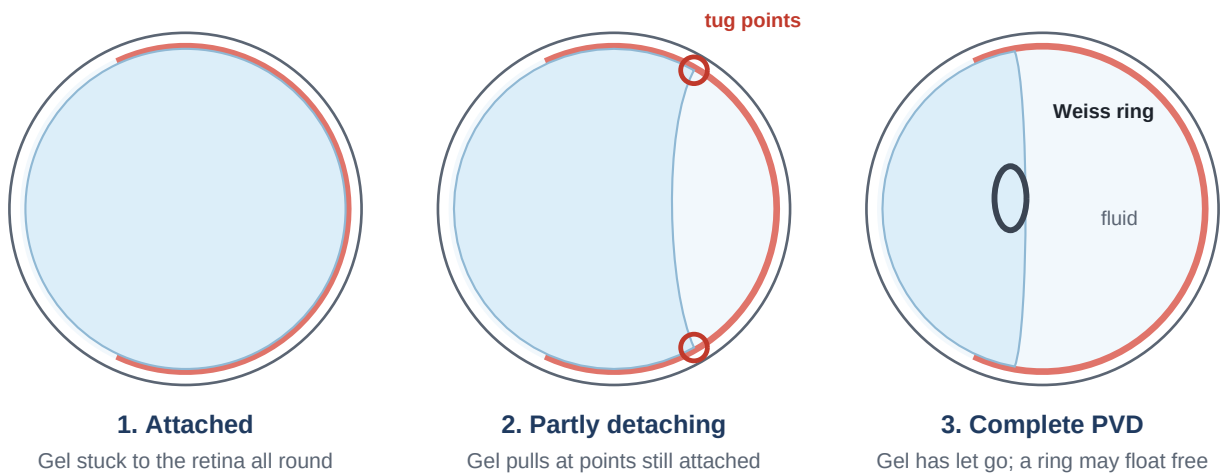
Why they drift and dart

The vitreous is a gel, so when the eye moves, the clumps swirl and then slowly settle. That's why floaters seem to follow your gaze and slide away when you look at them. Gravity pulls them down over time, which is one reason they often become less noticeable.

CHAPTER 3

Posterior vitreous detachment (PVD)

As the gel liquefies it also **shrinks**. Eventually it peels away from the retina at the back of the eye. This is called a **posterior vitreous detachment**, or PVD. Despite the scary word 'detachment', a PVD is **not** the same as a retinal detachment. It is a normal event that happens to most people: roughly half of people have one by their 60s, and most people by their 80s.



Red circles = where the gel can tug on the retina. This is when a tear can (occasionally) happen.

Figure 4. The three stages of a PVD. The process can take weeks, months or even years to complete.

What a PVD feels like

- A **sudden new floater** or a cluster of them, often a large ring, cobweb or 'fly' shape.
- Brief **flashes of light**, often at the edge of vision and more noticeable in the dark. These happen because the gel tugs on the retina, and the retina can only 'reply' by signalling light.
- Symptoms are often worst in the first few weeks, then gradually calm down.

Why PVD matters: the small risk of a retinal tear

In most people the gel lets go cleanly. But where it is stuck more firmly, it can pull a small **tear** in the retina. Studies suggest roughly **1 in 7 people** (about 14%) who see a doctor with sudden PVD symptoms have a tear, and the risk is higher when there are many new floaters, bleeding in the gel, or a drop in vision.

A tear can let fluid seep underneath the retina and lift it off: a **retinal detachment**. Tears found early are usually sealed with a quick laser or freezing treatment in clinic. That is why a **dilated eye check soon after new floaters** matters so much, and why some clinics book a follow-up check a few weeks later.

CHAPTER 4

Why did it move from one eye to the other, then come back?

Your dad's pattern, as described

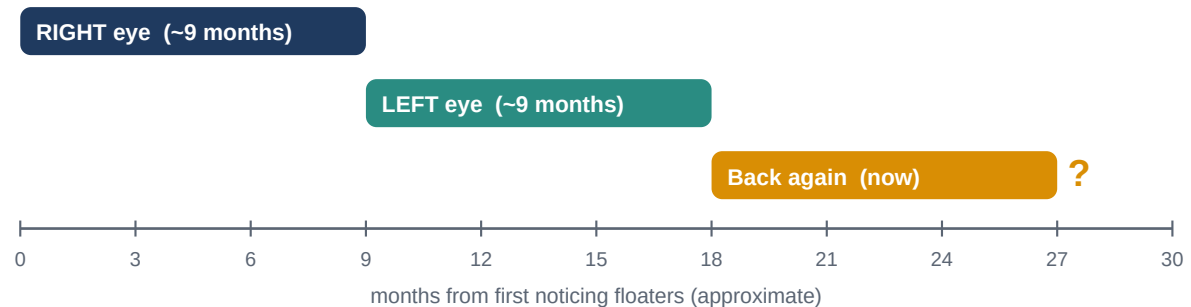


Figure 5. The pattern you described. Timings are approximate.

This pattern is not unusual, and there are several possible explanations. Only a dilated eye examination can tell which one applies to your dad, but here is what each one means:

Most likely: both eyes are ageing together

The gel in both eyes is the same age and changes on a similar timetable. It is very common for a PVD in one eye to be followed by a PVD in the other eye, often within a year or two. So right eye first, then left eye, fits normal ageing well.

Very common: they never really 'left', the brain tuned them out

Floater rarely disappear completely. Over months they sink below the line of sight and the brain learns to filter them out (called **neuroadaptation**). When the second eye got new floaters, attention switched there. Later, a change in lighting, tiredness, stress, new screens or simply noticing them again can bring the older ones 'back'.

Possible: a PVD happening in stages

A PVD is not always a single event. The gel can partly separate, pause for months, then finish separating, releasing new floaters each time. A new wave in an eye that settled before is worth a fresh check, because each new tug carries the small tear risk.

Less common but important: inflammation (uveitis)

Inflammation inside the eye can cause floaters that come and go and sometimes switch between eyes. Clues include redness, eye ache, sensitivity to light, or blurred vision, and conditions like arthritis, bowel disease or psoriasis in the person or family. Uveitis is treatable (often with steroid drops or tablets), but needs a doctor to diagnose it.

Less common: bleeding into the gel

Especially in people with diabetes or high blood pressure, small bleeds can cause 'smoky' or cobweb floaters. Good control of blood sugar and blood pressure matters here.

CHAPTER 5

Is it dangerous?

Usually not. The vast majority of floaters are a harmless nuisance. The danger is not the floater itself; it is what can occasionally happen at the **same time** (a retinal tear or detachment), or an underlying cause like inflammation or bleeding. Use this traffic-light guide:

GREEN: routine	Old, familiar floaters that are not changing. No flashes, no shadow, vision is normal. Action: mention at the next regular eye test (every 1-2 years; yearly over 70 or if diabetic).
AMBER: within days	A small number of new floaters, or old ones clearly returning, with no flashes, no shadow and normal vision. Action: book an optometrist appointment within a few days and ask for a dilated examination.
RED: same day	A sudden shower of floaters, flashes , a shadow or curtain , blurred or lost vision, a painful red eye, or floaters after an eye injury. Action: same-day urgent care: eye casualty, A&E / ER, or NHS 111.

Retinal detachment: what it looks like

A detached retina is painless, which is why people sometimes ignore it. The typical sequence is: flashes and new floaters, then a dark shadow in the side vision that grows over hours or days, sometimes described as a curtain, veil or wall of water. If the centre of the retina (the **macula**) is still attached when it is repaired, the chance of keeping good vision is much higher. **Speed really matters.**

Simple home check (optional)

Once a week, in good light, cover one eye and look at a fixed point such as a door frame. Notice whether any part of the surrounding view is missing, dark or wavy. Then repeat with the other eye. Any new gap or shadow is a RED symptom. This does not replace an eye exam, but it helps catch a change early.

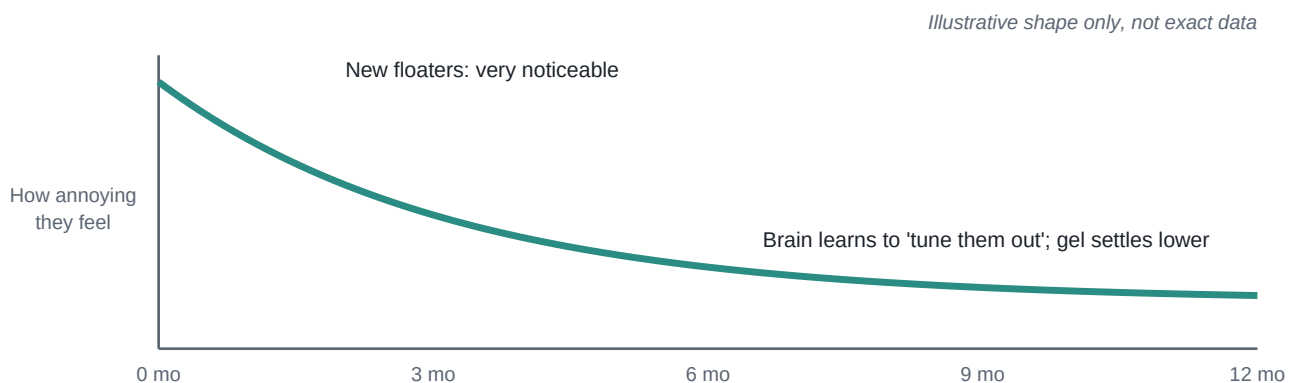


Figure 6. For most people, floaters feel worst at the start and fade into the background over months.

CHAPTER 6

How floaters are checked

Floaters cannot be properly assessed by looking at the front of the eye. The pupil needs to be widened with drops so the whole retina, including the far edges where tears usually happen, can be examined.

Test	What happens	What it shows
Dilated eye exam	Drops widen the pupil (vision is blurry and light-sensitive for 3-6 hours, so don't drive). The clinician looks inside with bright lights and lenses.	PVD, tears, bleeding, inflammation, detachment
OCT scan	A quick, painless camera scan, like an ultrasound using light. Nothing touches the eye.	Whether the gel has separated; detail of the retina
Retinal photos / wide-field imaging	Photos of the back of the eye.	A record to compare against next time
Eye ultrasound	A probe on the closed eyelid, used if the view is blocked (e.g. by blood).	Detachment behind a hazy view
Blood tests / referral	Only if inflammation is suspected.	Causes of uveitis

Who to see

- **Optometrist / optician:** the usual first stop, can do a dilated exam and OCT, and refer on if needed. Many UK high-street opticians offer free urgent appointments for new floaters and flashes (often called a minor eye conditions service).
- **Ophthalmologist (eye doctor):** hospital specialist; treats tears, detachments, uveitis, and does laser or surgery.
- **GP / family doctor:** helpful for diabetes, blood pressure, medicines and supplement safety, but cannot fully examine the retina.

CHAPTER 7

How are floaters treated? Can they be 'cured'?

The honest answer: for most people **no treatment is needed**, and the best 'cure' is time. There are two medical procedures for floaters that are severe and persistent and genuinely affect daily life (reading, driving, work). Both carry real risks, so they are only offered when the benefit clearly outweighs them.

	Watch and wait	YAG laser vitreolysis	Vitrectomy (surgery)
What it is	Regular checks while the brain adapts and floaters settle.	A laser breaks up or vaporises a large floater. Done in clinic, a few minutes.	The gel is removed through tiny ports and replaced with fluid. Done in an operating theatre.
How well it works	Most people are much less bothered within 3-12 months.	Mixed. In one US trial about half of treated patients felt a big improvement, versus about 1 in 10 with a sham. Works best for a single large ring (Weiss ring) away from the lens and retina.	The most effective option: floaters are removed in nearly everyone, and satisfaction is high.
Main risks	Missing a tear if red-flag symptoms are ignored.	Raised eye pressure, damage to the lens or retina (rare), floaters broken into smaller pieces.	Cataract (very common afterwards in people over 50), retinal detachment (about 1-2%), infection (rare), raised eye pressure.
Availability	Everyone.	Limited. Mostly private; health bodies such as the UK's NICE say the evidence is still limited.	Usually reserved for severe, long-lasting cases, or where there's bleeding or detachment.

Treatments for the underlying cause

- **Retinal tear:** laser or freezing (cryotherapy) to seal it, usually the same or next day, done in clinic.
- **Retinal detachment:** surgery (vitrectomy, a buckle, or a gas bubble). Urgent.
- **Uveitis:** steroid eye drops, injections or tablets, and treating any linked condition.
- **Bleeding from diabetes:** laser or injections to the retina, and good blood sugar control.

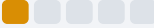
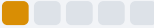
What does not work

There is no good evidence that eye exercises, eye massage, 'floater-dissolving' eye drops, homeopathic drops, or special diets remove floaters. Be wary of products that promise to 'dissolve' them.

CHAPTER 8

Vitamins & supplements: the evidence

You asked about vitamins, so here is a straight answer. **No supplement is proven to remove floaters.** A few have been tested in small studies with encouraging results, but the research is early and not strong enough for eye doctors to recommend them routinely. If your dad wants to try one, it is reasonable as long as it is safe for him, but it should never replace an eye exam.

Supplement	Evidence for floaters	What the research says	Safety notes
Combination supplement: L-lysine, vitamin C, zinc, grape seed extract, citrus bioflavonoids, hawthorn		A small 2021 randomised trial (around 60 people, 6 months) found treated people reported less floater disturbance and scans showed smaller floaters vs placebo. Promising, but one small study.	Hawthorn can interact with heart and blood-pressure medicines. Long-term high-dose zinc can lower copper.
Enzymes: bromelain (pineapple), papain (papaya), nattokinase (fermented soy)		One uncontrolled study from Taiwan (2019) reported floaters shrank in many people taking these enzymes for months. No placebo group, so the result is uncertain.	These can thin the blood. Avoid with aspirin, clopidogrel, warfarin, apixaban, rivaroxaban, or before surgery, unless a doctor agrees.
Hyaluronic acid or collagen supplements		No good evidence they reach the vitreous or affect floaters.	Generally safe but likely a waste of money for floaters.
Eye-health vitamins (AREDS2-type: vitamins C & E, zinc, copper, lutein, zeaxanthin)		Proven to slow macular degeneration in people who already have it. Not shown to help floaters.	Only worth taking if an optometrist recommends it for macular health.
Omega-3 (fish oil), vitamin D, B vitamins, multivitamins		Good for general health if the diet is lacking; no evidence for floaters.	Fish oil can mildly thin the blood at high doses.

Evidence bars: 0 = none; 1 = weak / indirect; 2 = one small controlled trial; 5 would be several large high-quality trials. Nothing reaches 3 yet.

Before starting ANY supplement

- Show the exact product to his **GP or pharmacist** and list every medicine he takes, especially blood thinners, heart or blood-pressure tablets, and diabetes medicines.
- Follow the dose on the label; more is not better. Stop 1-2 weeks before any surgery if advised.
- Give it a fair trial (the studies ran for about 3-6 months), and keep a symptom diary (page 14) to judge honestly whether it helps.
- Remember floaters often fade by themselves over the same timescale, so improvement may not be the supplement.

Food and lifestyle that support overall eye health

- **Leafy greens and colourful veg** (spinach, kale, peppers, sweetcorn) for lutein and zeaxanthin; **oily fish** twice a week; **citrus fruit and berries** for vitamin C; **nuts and seeds** for vitamin E and zinc.
- **Keep blood sugar and blood pressure well controlled:** this protects the retina's blood vessels and reduces bleeding.
- **Don't smoke:** smoking damages the blood vessels in the eye and raises the risk of several eye diseases.
- **Stay hydrated** and get regular eye tests. (Water doesn't dissolve floaters, but dehydration doesn't help anything.)
- **Wear eye protection** for DIY, gardening and sport to avoid injuries.

An example eye-friendly day

Breakfast: porridge with berries and a handful of walnuts or seeds; an orange.

Lunch: spinach or kale salad with eggs, peppers and sweetcorn, olive-oil dressing.

Dinner: salmon, mackerel or sardines (or beans and lentils) with broccoli and sweet potato.

Snacks: kiwi, carrots and hummus, a small handful of almonds or pumpkin seeds.

This supports the retina and general health. It won't dissolve floaters, but it is the one 'vitamin plan' with no downside.

CHAPTER 9

Everyday tips for living with floaters

Move them out of the way

Look up and down (not side to side) a couple of times. This swirls the gel and can shift a floater out of the centre of vision.

Soften bright backgrounds

Use dark mode on phones and computers, lower screen brightness, and avoid staring at white pages or bright skies. Floaters show up most against bright, plain backgrounds.

Wear sunglasses outdoors

Polarised or tinted lenses reduce glare and make floaters far less visible on sunny days.

Adjust lighting

Use warm, indirect lamps rather than harsh overhead light. A reading lamp to the side works better than one behind.

Stop 'checking' for them

The more you look for floaters, the more the brain notices them. Try not to test your vision against the sky or white walls repeatedly.

Manage stress and tiredness

Floaters often seem worse when tired or anxious. Good sleep and breaks from screens help the brain filter them out.

Driving

Floaters alone don't usually affect the legal right to drive, but if they genuinely block vision, or there is any shadow or loss of vision, stop driving and get checked.

The emotional side is real too

Floaters can be surprisingly distressing: some people feel anxious, frustrated or low, especially at the start. That's normal and it usually eases as the brain adapts. If they are seriously affecting mood, sleep or work after several months, tell the eye doctor. That is exactly the situation in which treatment options are discussed.

Myths vs facts

Myth	Fact
"Floaters mean I'm going blind."	Almost always false. Most floaters are harmless ageing changes. The rare dangerous causes have clear warning signs (page 7).
"If they went away, the problem is solved."	They usually just sink and get filtered out. Any NEW wave still needs a check.
"Eye drops can dissolve them."	No eye drop is proven to do this.
"Screens cause floaters."	Screens don't cause them; bright white screens just make them easier to see.
"A PVD is a retinal detachment."	No. A PVD is the gel separating (normal). A retinal detachment is the retina itself lifting off (an emergency).

CHAPTER 10

Questions to ask at the appointment

Take this page along. Tick off each question as it's answered.

- ☐ Did you dilate my pupils and examine the whole retina in **both** eyes?
- ☐ Do I have a posterior vitreous detachment? Is it complete or partial, in which eye(s)?
- ☐ Is there any **tear, hole, bleeding** or **inflammation**?
- ☐ Why do you think the floaters moved from my right eye to my left and then came back?
- ☐ Do I need a follow-up check, and when?
- ☐ Am I at higher risk of a retinal detachment (short-sighted, past surgery, family history)?
- ☐ Could any of my health conditions or medicines be playing a part?
- ☐ Are my floaters bad enough to consider laser or surgery? What are the risks for someone like me?
- ☐ Is there any supplement you'd recommend or advise against for me?
- ☐ Exactly which symptoms should make me come back urgently, and where do I go out of hours?

Information to bring

- The timeline: when floaters started in each eye, and what they looked like (dots, strands, ring).
- Any flashes, shadows, redness, pain or light sensitivity, even if brief.
- Glasses prescription, past eye surgery or injuries, and a list of medicines and supplements.
- Health conditions (diabetes, blood pressure, arthritis, bowel disease, psoriasis) and family eye history.
- Someone to drive home afterwards: dilating drops blur vision for several hours.

Filling this in weekly makes it much easier to spot a real change and to describe things accurately to the eye doctor.

[illegible]

Any 'Y' in the flashes or shadow columns, or 'MORE' that came on suddenly = same-day check.

REFERENCE

Glossary and sources

Vitreous	The clear gel filling the main chamber of the eye.
Floater	A shadow cast on the retina by a clump in the vitreous.
Syneresis	The natural liquefying of the vitreous gel with age.
PVD (posterior vitreous detachment)	The vitreous gel peeling away from the retina. Common and usually harmless.
Weiss ring	A ring-shaped floater left where the gel was attached around the optic nerve. A sign of complete PVD.
Retina	The light-sensitive layer at the back of the eye.
Macula	The centre of the retina, used for reading and detail.
Retinal tear / detachment	A break in the retina / the retina lifting off the back wall of the eye. Urgent.
Photopsia	The medical term for seeing flashes of light.
Uveitis	Inflammation inside the eye; can cause floaters, pain, redness, light sensitivity and blur.
Neuroadaptation	The brain learning to ignore a constant visual signal, such as a floater.
YAG vitreolysis	Laser treatment to break up a floater.
Vitrectomy	Surgery to remove the vitreous gel.
OCT	Optical coherence tomography: a painless light-based scan of the retina.

Where this information comes from

- NHS (nhs.uk): 'Floaters and flashes in the eyes' and 'Detached retina'.
- Royal College of Ophthalmologists and College of Optometrists guidance on flashes and floaters.
- American Academy of Ophthalmology (aao.org): 'What are floaters and flashes?' and PVD patient information.
- Hollands H. et al., JAMA 2009: systematic review of retinal tear risk in acute PVD (about 14%).
- Shah C.P. & Heier J.S., JAMA Ophthalmology 2017: randomised trial of YAG laser vitreolysis for Weiss ring floaters.
- NICE interventional procedures guidance on laser vitreolysis for floaters.
- Ankamah E. et al., 2021: randomised trial of a combination nutritional supplement for vitreous floaters.
- Horng C.T. et al., 2019: uncontrolled study of oral enzymes (bromelain, papain, nattokinase) for floaters.
- AREDS / AREDS2 studies (US National Eye Institute) on vitamins for macular degeneration.

Important

This guide is for general education and to help you have a better conversation with a professional. It cannot diagnose your dad's eyes. Statistics are approximate and research changes over time. If in doubt, get checked. With eyes, it's always better to be seen and reassured than to wait.